

NOMENCLATURE/TERMINOLOGY

The term **vitiligo (non-segmental)** is now preferred to describe all forms of vitiligo except segmental vitiligo, based on current consensus. This terminology replaces the previous umbrella term "vitiligo" to reduce confusion in clinical, research, and practical settings.

- **Segmental vitiligo (SV):** Characterized by a clinically unilateral, segmental distribution of depigmented macules and patches.
- **Mixed vitiligo:** Diagnosed when segmental vitiligo coexists with vitiligo (non-segmental).
- **Focal vitiligo:** Refers to localized depigmented macules without a segmental or generalized pattern. It is classified under **undetermined/unclassified vitiligo** until further clinical evolution is observed (typically after 1–2 years of follow-up).
- **Pure mucosal vitiligo** (involving only one mucosal site) and long-standing focal vitiligo that does not progress may remain in the **unclassified** category if they do not meet criteria for other types.

The updated classification is summarized in **Table 2**.

An international consensus has also established standardized definitions for **vitiligo activity**, incorporating patient-reported outcomes and clinical validation. These definitions help characterize disease behavior such as rapidly spreading or progressive disease (**Table 3**).

INITIAL PATIENT ASSESSMENT

A comprehensive initial consultation—recommended to last **30–45 minutes**—is essential for effective management. This allows sufficient time to:

- Discuss the pathogenesis and prognosis of vitiligo
- Explain treatment options and rationale
- Address psychosocial impact and patient expectations

- Review practical aspects of therapy

The vitiligo-specific assessment should include:

- Detailed medical and family history (including autoimmune conditions)
- Current medications
- Onset, progression pattern, and duration of lesions
- Clinical examination with documentation (e.g., dermoscopy, body mapping)
- **Clinical photography or digital imaging** for monitoring disease progression and treatment response

A structured checklist of key assessment components is provided in **Table 4**.

In alignment with the core therapeutic goal of **preventing further melanocyte damage**, the assessment must also guide individualized, patient-centered management strategies.